NCCP 00250 · VERSION 9

Your treatment, explained

Adjuvant high-risk node-positive or node-negative operable breast cancer · NCCP 00250 v9 · Document revised 4 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Docetaxel + cyclophosphamide (TC)

21-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Report new breathing symptoms, severe diarrhoea, inability to drink, painful swollen limbs, unusual bleeding, or worsening numbness. Tell staff immediately about infusion-site pain or an allergic reaction.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Open your CUH contact / treatment card

Sources: nccp · hse

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- The phases and how they connect
- Your 21-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Tablets, visits and practical planning
- Why this treatment?
- Checks across treatment
- Surgery, radiotherapy and later treatment
- Supportive medicines
- Trial results
- Questions to ask
- Sources

UNDERSTAND YOUR TREATMENT

How the medicines work

Treatment after surgery to reduce the chance of breast cancer returning.

TAXANE CHEMOTHERAPY

Docetaxel



Disrupts the cell's division machinery

Docetaxel stabilises tiny structures called microtubules. Cells need these to separate their chromosomes during division; disrupting the process can stop cancer cells multiplying. Healthy dividing cells are also affected.

Microtubules stabilised → Division disrupted → Cell growth affected

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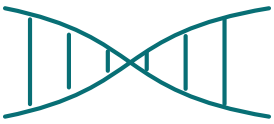
What to watch for

Take prescribed steroid premedication. Report fever, lasting numbness, swelling, sore nails or infusion symptoms.

Sources: nccp

ALKYLATING CHEMOTHERAPY

Cyclophosphamide



Damages DNA needed for growth

Cyclophosphamide is activated in the body and damages DNA. This helps stop dividing cancer cells but can also affect blood-cell production and reproductive tissues.

Drug activated → DNA damage → Cell division affected

What to watch for

Use the prescribed anti-sickness and hydration plan. Report fever, painful urination or blood in the urine.

Sources: nccp

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Explore your existing anatomy and side-effect map

THE BIGGER TREATMENT PLAN

The phases and how they connect

21-DAY INTERVAL

TC

4–6 cycles

Docetaxel and cyclophosphamide on Day 1 every 21 days for four to six cycles. The team confirms the planned course before treatment. Radiotherapy, when indicated, follows chemotherapy.

The treatment phase determines the medicines, supportive prescription and tests. A phase change does not reset previous drug exposure or cardiac monitoring.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative milestones, not equal time intervals. The clinical team owns booking, result review and action on overdue tests. This is an educational map, not an automated appointment or result tracker.

Check	Before starting	Early cycles	Later cycles / transition	Next treatment / completion	Follow-up
Blood tests	FBC, kidney and liver profile	Repeat as clinically indicated; confirm the unit's pre-treatment test plan	Review testing needs at treatment changes	Follow active medicines and abnormalities	Follow unresolved abnormalities
Echo / heart	History / symptoms review	No routine TC-specific echo	Investigate if clinically indicated	Follow any cardiac concern	Individual plan
ECG	ECG only for clinical indications or interacting medicines; not a routine TC-cycle requirement.	Symptoms / risk-based repeat	Review if cardiac concern	ECG and echo answer different questions	Clinical indication
Cancer imaging	Record baseline extent as appropriate	Clinical response / symptoms review	Review new symptoms; this TC regimen is adjuvant	Arrange radiotherapy or further treatment if indicated	Planned breast imaging; no routine CT/PET
Markers / bone health	Tests selected for active medicines	No routine tumour-marker clearance	No routine blood tumour markers when well	DEXA if endocrine/ bone-risk indication	Follow the endocrine and bone-health plan

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00250 v9 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · NICE NG101 · bone health and follow-up · ESMO metastatic breast cancer guideline · 2026

THE WHOLE TREATMENT JOURNEY

Docetaxel + cyclophosphamide (TC): treatment and checks

Adjuvant high-risk node-positive or node-negative operable breast cancer

Docetaxel and cyclophosphamide on Day 1 every 21 days for four to six cycles. The team confirms the planned course before treatment. Radiotherapy, when indicated, follows chemotherapy.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

Treatment, tests and reviews across the course

Care / check	Before starting	Early cycles	Later cycles / transition	Next treatment / completion	Follow-up
Cancer medicines NCCP 00250	Agree aim, indication and medicines	Docetaxel + cyclophosphamide (TC) starts	Review response, toxicity and exposure	Complete planned course / next decision	Ongoing review / subsequent plan
Surgery EARLY	Adjuvant: surgery already completed	No automatic operation during cycles	Review postoperative recovery and symptoms	Surgery was completed before this adjuvant regimen	Pathology and recovery guide next steps
Radiotherapy EARLY	Assess indication	Usually after cytotoxic chemotherapy	Arrange sequential timing	Radiation team confirms start	Review effects / follow-up
Blood tests NCCP 00250	FBC, kidney and liver profile	Repeat as clinically indicated; confirm the unit's pre-treatment test plan	Review testing needs at treatment changes	Follow active medicines and abnormalities	Follow unresolved abnormalities
Echo / heart NCCP 00250 · ESC	History / symptoms review	No routine TC-specific echo	Investigate if clinically indicated	Follow any cardiac concern	Individual plan
ECG NCCP 00250	ECG only for clinical indications or interacting medicines; not a routine TC-cycle requirement.	Symptoms / risk-based repeat	Review if cardiac concern	ECG and echo answer different questions	Clinical indication
Cancer imaging NCCP 00250	Record baseline extent as appropriate	Clinical response / symptoms review	Review new symptoms; this TC regimen is adjuvant	Arrange radiotherapy or further treatment if indicated	Planned breast imaging; no routine CT/PET
Markers / bone health NCCP 00250	Tests selected for active medicines	No routine tumour-marker clearance	No routine blood tumour markers when well	DEXA if endocrine/ bone-risk indication	Follow the endocrine and bone-health plan
Endocrine / bone BONE · EARLY	Confirm hormone receptors	Plan endocrine therapy when appropriate	Separate from this chemotherapy prescription	Start/continue per MDT; DEXA when indicated	Risk-based bone review

What each check is for

1

Before first dose

Baseline

Baseline blood count, kidney and liver tests. The team arranges repeat testing as clinically indicated and confirms which results are needed before treatment.

Ask / act: Confirm tests and consent before prescribing.

Sources: NCCP 00250

2

Heart-monitoring plan

Baseline → cycles → after treatment

TC contains no anthracycline or HER2-targeted antibody. There is no routine three-month echo programme for TC alone; arrange cardiac tests for symptoms, prior exposure or other clinical indications.

Ask / act: Name the team responsible for booking and reviewing results.

Sources: NCCP 00250 · ESC

3

Treatment-phase review

Before each transition

Docetaxel and cyclophosphamide on Day 1 every 21 days for four to six cycles. The team confirms the planned course before treatment. Radiotherapy, when indicated, follows chemotherapy.

Ask / act: Confirm active medicines, support script and the next test checkpoint.

Sources: NCCP 00250

4

Response and follow-up

Planned review points

For early breast cancer, breast imaging and clinical review follow the agreed pathway. Routine CT/PET scans and blood tumour markers are not used to look for distant recurrence when there are no symptoms or clinical concerns.

Ask / act: Review symptoms and imaging together; do not change treatment on tumour markers alone.

Sources: NCCP 00250

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-23 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.17

TC

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

TC

Day of cycle	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21
Chemotherapy · Day 1	IV																				
Administration reaction	Report during or after any visit																				
Nausea / appetite	May occur early or later																				
Blood counts / infection	Risk varies; fever needs prompt advice																				
Fatigue / mouth changes	Variable timing and severity																				

Day of cycle	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21
Nerves / skin / nails	Can build across doses																				

No dates to enter: The chart explains the pattern. Your team books actual appointments and reviews results.

Treatment delays: A delayed dose does not automatically delay an echo or other test that is already due.

Phase changes: Do not change the schedule using this educational map.

> **Why these bands stay broad:** These broad symptom bands are educational possibilities, not measured risk curves. Infusion days are the planned schedule; delays require a new clinical plan. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00250 v9 · official regimen ↗️

> **There are no guaranteed "safe days".** Fever, shivers, sudden illness, severe breathing changes or chest pain need prompt assessment on any day.

Read the whole cycle as text

Day 1–21: Your current reference cycle

Docetaxel and cyclophosphamide on Day 1 every 21 days for four to six cycles. The team confirms the planned course before treatment. Radiotherapy, when indicated, follows chemotherapy. Follow the team's appointment and medicine instructions; contact them about missed doses or new symptoms.

PRACTICAL TREATMENT GUIDE

Your practical treatment plan

Docetaxel and cyclophosphamide on Day 1 every 21 days for four to six cycles. The team confirms the planned course before treatment. Radiotherapy, when indicated, follows chemotherapy.

STEP 1

Before the first visit

Blood tests, relevant cardiac assessment, medicines review, fertility discussion and the consent conversation come first.

STEP 2

Plan venous access

A cannula, PICC or implanted port may be used. If a port is chosen, a trained team inserts it in a separate procedure with local anaesthetic and sometimes sedation; the team explains fasting, blood-thinner and wound instructions.

STEP 3

Infusion day

Staff check the line, give prescribed premedicines and treatment, and observe for reactions. Tell them immediately about burning, pain, rash or breathlessness.



STEP 4

Home and next review

This regimen does not routinely need a take-home infusion pump. Staff disconnect the infusion; a port is de-accessed when appropriate. Keep line-care and next-visit instructions.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Visit time and transport

First visits can take longer because of loading doses, premedicines and observation. Ask the unit about expected time, transport and any escort needed after sedation or line insertion.

Phase-specific visit pattern

Infusion and observation times differ by drug and first versus later doses; the unit confirms the full visit duration, including blood review and preparation.

Open practical treatment guide · Printable practical guide

Sources: NCCP 00250 v9 · official regimen · eviQ · implanted port patient information · eviQ · PICC patient information. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 The correct phase prescription

TC uses moderate-risk antiemetic support.

2 Premedicines and growth factors

Docetaxel dexamethasone is taken for three days starting the day before treatment. Ask the team to reconcile this with antiemetic steroids.

3 Tests and appointments

The team should name the booking owner, due window and person reviewing each required test. Tell them if an expected appointment has not arrived.

4 **After chemotherapy**

Discuss recovery, fertility, rehabilitation and the next treatment or surveillance step.

Sources: nccp · antiemetic

TREATMENT REFERENCE · 01

Why this treatment?

TC is an anthracycline-free chemotherapy option after surgery. The team weighs recurrence risk and cancer biology against benefits and side effects. Avoiding an anthracycline can matter for some people, but TC still needs blood tests, infection precautions and a docetaxel steroid plan.

Your treatment setting

Adjuvant high-risk node-positive or node-negative operable breast cancer

Sources: nccp

The sequence

Docetaxel and cyclophosphamide on Day 1 every 21 days for four to six cycles. The team confirms the planned course before treatment. Radiotherapy, when indicated, follows chemotherapy.

Sources: nccp

Alternatives and your priorities

Ask what alternatives fit your cancer, what the treatment aims to achieve, and how its practical demands fit your life. The team can discuss personal benefits and risks.

Sources: nccp

Checks across treatment

Your team arranges, reviews and acts on tests; an overdue result needs follow-up.

Blood tests

Baseline blood count, kidney and liver tests. The team arranges repeat testing as clinically indicated and confirms which results are needed before treatment.

Sources: nccp

Heart monitoring

TC contains no anthracycline or HER2-targeted antibody. There is no routine three-month echo programme for TC alone; arrange cardiac tests for symptoms, prior exposure or other clinical indications.

Sources: nccp

ECG

ECG only for clinical indications or interacting medicines; not a routine TC-cycle requirement.

Sources: nccp

Cancer scans

For early breast cancer, breast imaging and clinical review follow the agreed pathway. Routine CT/PET scans and blood tumour markers are not used to look for distant recurrence when there are no symptoms or clinical concerns.

Sources: early

TREATMENT REFERENCE · 03

Surgery, radiotherapy and later treatment

Surgery → this adjuvant chemotherapy → radiotherapy if indicated. The surgical team decides lumpectomy versus mastectomy; the radiation plan depends on surgery, pathology and individual risk.

Hormone-receptor-positive disease

Endocrine treatment is considered when appropriate for the cancer biology and setting. It has its own schedule and monitoring; it is not automatically part of this chemotherapy prescription.

Sources: early · metastatic

Bone health

DEXA is linked to an aromatase inhibitor, ovarian suppression, treatment-induced menopause or other bone risk—not automatically to every chemotherapy course. The endocrine team confirms baseline testing and risk-based repeat scans.

Sources: bone

Markers and follow-up

After local treatment, the team plans mammography of remaining breast tissue. A mastectomy side does not need routine mammography. Report new symptoms rather than waiting for a scan.

Sources: bone

Genomic tests

Where appropriate, the MDT may discuss a validated genomic test to inform the chemotherapy decision. This regimen card does not calculate an Oncotype result or select treatment from a score.

Sources: early

UNDERSTANDING THE EVIDENCE

Why this treatment may be chosen

TC is an anthracycline-free chemotherapy option after surgery. The team weighs recurrence risk and cancer biology against benefits and side effects. Avoiding an anthracycline can matter for some people, but TC still needs blood tests, infection precautions and a docetaxel steroid plan.

Show trial results and what the numbers mean

Adjuvant high-risk node-positive or node-negative operable breast cancer

- Pembrolizumab + chemotherapy
- Placebo + chemotherapy

Your team can discuss expected benefit and alternatives in the context of your own cancer and priorities.

Sources: nccp · early

PREPARE FOR THE CONVERSATION

Questions worth asking

What changes between phases?

Docetaxel and cyclophosphamide on Day 1 every 21 days for four to six cycles. The team confirms the planned course before treatment. Radiotherapy, when indicated, follows chemotherapy.

When are heart checks due?

No routine fixed-cycle echo or ECG is mandated for this regimen alone; assess symptoms and individual cardiac risk. Test timing follows the treatment, clinical findings and local pathway.

Where do surgery and radiotherapy fit?

In adjuvant care, radiotherapy usually follows chemotherapy when indicated; endocrine therapy follows its own receptor-specific plan.

Do I need routine CT scans or tumour markers?

Early breast follow-up uses planned breast imaging. Routine CT / PET, blood tumour markers or other distant-recurrence screening are not recommended without symptoms or a clinical indication. After local treatment: annual mammography of remaining breast tissue; additional imaging when indicated.

Does a port mean a pump at home?

No. This regimen has no routine take-home infusion pump. A port is an access device; the team removes the needle after the visit when appropriate and arranges device removal separately.

Who arranges the scans and tests?

Your treating service should book the planned checks, review the results and explain any change. If an expected appointment is missing, contact the team; this map does not book tests or confirm that they have happened.

Why might this treatment be selected for me?

TC is an anthracycline-free chemotherapy option after surgery. The team weighs recurrence risk and cancer biology against benefits and side effects. Avoiding an anthracycline can matter for some people, but TC still needs blood tests, infection precautions and a docetaxel steroid plan.

Who arranges the scans and tests?

Your treating service should book the planned checks, review the results and explain any change. If an expected appointment is missing, contact the team; this map does not book tests or confirm that they have happened.

> **Try explaining the plan back:** Can you explain which medicines you are receiving now? What symptoms mean you should call urgently? Which blood tests, scans or heart checks come next, and who arranges them?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00250 v9. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00250 v9 · official regimen
- ESMO 2024 early breast cancer guideline · treatment and follow-up
- ESC 2022 cardio-oncology guideline · cardiac surveillance and risk
- HSE · chemotherapy side effects and urgent advice
- NICE NG101 · bone health and follow-up
- NOGG · section 7, aromatase inhibitors and fracture risk
- eviQ · implanted port patient information
- eviQ · PICC patient information
- NCCP NCIS antiemetics v8 (2025), Table 1
- ASCO · biomarkers in metastatic breast cancer
- ESMO metastatic breast cancer guideline · 2026
- Regimen selection and source references

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-10-23.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6353/250_DOCEtaxel-cycloPHOS_TC.pdf

https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext

https://academic.oup.com/eurheartj/article/43/41/4229/6673995

https://www.nice.org.uk/guidance/ng101/chapter/Recommendations

https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext

https://www.eviq.org.au/p/507

https://www.eviq.org.au/p/543

https://www2.hse.ie/conditions/chemotherapy/side-effects/

https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk

https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf

https://ascopubs.org/doi/10.1200/JCO.2015.61.1459